

SOP / OPERATIONAL MANUAL

STANDARD OPERATING PROCEDURE &
OPERATIONAL REFERENCE

Well Spring Intervention LLC

LEVEL 3 SUPERVISED RESIDENTIAL GROUP HOME

EMPOWERMENT · GROWTH · FREEDOM · HEALTH · WHOLENESS ·
HEALING



SOP & Operational Manual

STANDARD OPERATING PROCEDURES, PROTOCOLS & FORMS

ABOUT THIS MANUAL

Document Overview

This manual (Rev. 2.9, July 2026) is the official Standard Operating Procedures and Operational Reference for **Well Spring Intervention LLC**, a Level 3 Supervised Residential Group Home serving children and adolescents with mental health and behavioral challenges. It establishes the policies, actionable workflows, and reference forms governing the daily operation of the program. All staff are required to read, acknowledge, and follow these protocols exactly as written.

Population Served. Children & Adolescents — Mental Health / Behavioral Challenges.

Service Type. Level 3 Supervised Residential Group Home.

Effective Date. July 2026.

Document Owner. Executive Director & Qualified Professional (QP).

Document ID. Doc. WSI-SOP-001, Rev. 2.9 (RMDM-Compliant). *Versioning is private — this information does not appear on the public-facing cover.*

Regulatory Framework.

This manual is governed by, and operates in conformance with, the following authorities: **10A NCAC 27G .5600** (residential facility standards); **10A NCAC 27G .0104** (Qualified Professional credentials); **NC Medicaid CCP 8C** (residential services); **10A NCAC 27T** (Rule 108 — clinical record content); **NC DHSR** facility licensure; **LME/MCO Tailored Plan** requirements; **NCDHHS Records Management and Documentation Manual** (RMDM, Effective July 8, 2025); **HIPAA** Privacy, Security, and Breach Notification Rules; **42 CFR Part 2** (confidentiality of SUD records); **HITECH Act**; **NCGS Chapter 66, Article 40** (NC Uniform Electronic Transactions Act); and the federal **E-SIGN Act** (15 U.S.C. § 7001 et seq.).

Revision Lineage.

Rev. 2.0 (Jul 2026) reorganized the manual to full RMDM compliance. Rev. 2.1 added §10.7 Electronic Signatures (NCGS Ch. 66 Art. 40 — NC UETA) with safeguards and system-unavailability procedures. Rev. 2.2 clarified the organizational reporting structure in §1.4, establishing the QP as reporting to the Clinical Director and providing recurring compliance reports via new §1.4(a). Rev. 2.3 refined QP responsibilities to explicitly include scheduling of clinical services, assessments, and PCPs in §1.4. Rev. 2.4 added §1.4(b) QP Credentialing Requirements per 10A NCAC 27G .0104. Rev. 2.5 corrected §1.4(b) to recognize BOTH acceptable QP pathways: Pathway 1 (master's degree + recognized NC credential + 1 year post-master's supervised MH/DD/SA experience) and Pathway 2 (bachelor's degree + 2 years full-time pre- or post-bachelor's supervised MH/DD/SA experience). Rev. 2.6 added a symbolic cover illustration evoking empowerment, growth, freedom, health, wholeness, and healing. Rev. 2.7 redesigns the cover as a full-bleed brand illustration suitable for reuse across company sites and publications, and relocates the descriptive cover content to this About This Manual page. Rev. 2.8 removes all versioning information (Doc ID, Revision number, RMDM-Compliance designation, Owner) from the public-facing cover so the cover can serve as a clean brand asset; versioning information remains accessible internally via this About This Manual page, the body page headers, the Version History table in Part 3, Form 6, and the PDF metadata. Rev. 2.9 refreshes the cover artwork with lush green leaves and an explicit well-spring in the foreground to complete the symbolic narrative of wellspring, growth, health, and flourishing; adds Protocol 22 (Daily Workflow Schedules for All

Personnel) covering QP, AP, DCP, House Manager, Awake Overnight, RN, and Billing Coordinator shift routines; and converts all nine forms in Part 3 to interactive AcroForm fillable PDF fields with a Form Properties banner declaring each form printable, copyable, sharable, editable, and fillable. A complete revision history appears in Part 3.

Cover Artwork.

The cover illustration is the official brand visual of Well Spring Intervention LLC. It depicts a stylized tree-human figure with lush green leaves flourishing toward a warm sunrise over calm water, with a clear well-spring bubbling up in the foreground directly before the tree. The composition completes the program's symbolic narrative: the well-spring is the source of renewal, the green leaves embody growth and flourishing, and the sunrise promises a new day. The image symbolizes the program's commitment to empowerment, growth, freedom, health, wholeness, and healing, and is approved for reuse across company websites, publications, and collateral materials. A high-resolution copy is available alongside this manual for that purpose.

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Table of Contents

This manual (Rev. 2.9, July 2026) is organized into three parts and is fully compliant with the NCDHHS Records Management and Documentation Manual (RMDM, Effective July 8, 2025). Part 1 establishes foundational policies and compliance obligations across eleven sections, including new chapters on privacy/confidentiality and an expanded Medicaid documentation section. Rev. 2.1 added §10.7 Electronic Signatures (referencing NCGS Chapter 66, Article 40 — NC UETA) with explicit safeguards and system unavailability procedures. Rev. 2.2 clarified the organizational reporting structure in §1.4, establishing the QP as reporting to the Clinical Director, supervising staff per the Clinical Director's direction, and providing recurring compliance reports via new §1.4(a). Rev. 2.3 refined the QP responsibilities to explicitly include scheduling of clinical services, assessments, and PCPs in §1.4. Rev. 2.4 added §1.4(b) QP Credentialing Requirements per 10A NCAC 27G .0104. Rev. 2.5 corrects the §1.4(b) QP credentialing framework to recognize BOTH acceptable pathways: Pathway 1 (master's degree + recognized NC credential + 1 year post-master's supervised MH/DD/SA experience) and Pathway 2 (bachelor's degree + 2 years full-time pre- or post-bachelor's supervised MH/DD/SA experience); §2.2 QP bullet updated to match. Rev. 2.6 added a symbolic cover illustration evoking empowerment, growth, freedom, health, wholeness, and healing — visually framing the trauma-informed, restorative mission of the program. Rev. 2.7 redesigns the cover as a full-bleed brand illustration suitable for reuse across company sites and publications, with descriptive cover content relocated to the About This Manual page (p. 2). Rev. 2.8 removes all versioning information from the public-facing cover (Doc ID, Revision number, RMDM-Compliance designation, Owner); versioning remains private and is accessible only via internal surfaces (PDF metadata, About This Manual page, body page headers, Version History table, Form 6). Rev. 2.9 refreshes the cover artwork with lush green leaves and an explicit well-spring in the foreground to complete the symbolic narrative of wellspring, growth, health, and flourishing; adds Protocol 22 (Daily Workflow Schedules for All Personnel) covering QP, AP, DCP, House Manager, Awake Overnight, RN, and Billing Coordinator shift routines; and converts all nine forms in Part 3 to interactive AcroForm fillable PDF fields with a Form Properties banner declaring each form printable, copyable, sharable, editable, and fillable. Standalone fillable copies of all forms are also available in the /download/forms/ directory. Part 2 details twenty-two step-by-step workflows that govern daily operations, including protocols for service orders/authorizations and record management/disclosure accounting. Part 3 provides nine customized forms and logs, including the Full Service Note Template, Comprehensive Clinical Record Content Checklist, and Accounting of Disclosures Log.

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PART 1

Standard Operating Procedures

These eleven SOP sections define the foundational policies, governance, staffing, clinical, privacy, and compliance obligations that govern every aspect of program operation. Version 2.0 expands coverage to full RMDM (Effective July 8, 2025) compliance, including record retention, CDW data reporting, comprehensive clinical record content, CCA requirements, service orders/authorizations, and privacy/confidentiality under HIPAA and 42 CFR Part 2. Every staff member is responsible for knowing and adhering to these policies.

1. Agency Overview & Governance

Reference: 10A NCAC 27G .0100; NC Medicaid Managed Care Tailored Plan Requirements; RMDM Chapter 1 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · §1

1.1 Mission Statement. Well Spring Intervention LLC is dedicated to providing trauma-informed, high-quality supervised living environments for youth with severe emotional disturbances (SED), ensuring their safety and clinical growth. Our program philosophy is rooted in the belief that every child deserves a stable, predictable, and nurturing environment in which healing can occur. All staff actions, decisions, and interventions must align with this mission and reflect our core values of safety, dignity, accountability, and growth.

1.2 Licensing & Credentialing. The facility operates under a valid NC DHSR license (Level 3 Supervised Residential Group Home) and is credentialed as an In-Network Provider with the regional LME/MCO/Tailored Plan. The Executive Director maintains the original license on site, posts a current copy in a public area of the facility, and renews it prior to expiration. Any change in ownership, capacity, population served, or physical location requires prior written approval from DHSR and notification to the LME/MCO.

1.3 Corporate Compliance. Well Spring Intervention LLC maintains a Corporate Compliance Plan to prevent Medicaid fraud, waste, and abuse. All staff receive compliance training upon hire and annually thereafter. The Compliance Officer receives reports, investigates concerns, and coordinates with the LME/MCO and Medicaid Fraud Investigations Unit. Retaliation against good-faith reporters is strictly prohibited and itself constitutes a reportable offense.

1.4 Organizational Structure. The Executive Director holds overall authority and accountability. The **Clinical Director** is a licensed clinical professional with overall responsibility for the clinical program, including setting the clinical vision, approving clinical policies, and providing direction to clinical leadership. The **Qualified Professional (QP)** reports to the Clinical Director and is responsible for scheduling clinical services, assessments, PCPs, and day-to-day supervision of Associate Professionals (APs) and Direct Care Professionals (DCPs) **according to the direction of the Clinical Director.** The QP supervises staff in accordance with the Clinical Director's clinical guidance, programmatic priorities, and performance expectations, and shall escalate clinical concerns, staffing issues, and quality-of-care matters to the Clinical Director in a timely manner. An On-Call QP is available 24/7/365 for clinical decision-making. All staff report incidents, concerns, and operational needs through the documented chain of command.

1.4(a) QP Compliance Reporting to the Clinical Director.

In addition to clinical supervision, the QP shall provide **compliance reports** to the Clinical Director on a recurring basis, ensuring the Clinical Director has timely visibility into the facility's regulatory and documentation posture.

These reports include: (a) **monthly** summaries of service-note audit findings, late entries, and alterations (per §10.1, §10.3, §10.5); (b) **monthly** IRIS incident-report status including any restraint events, restrictive interventions, and follow-up actions completed (per §5.4, §8); (c) **quarterly** Clinical Record Content Checklist audit results using Form 8, with corrective-action plans for any deficiencies; (d) **quarterly** Accounting of Disclosures review using Form 9, including any 42 CFR Part 2 disclosures; (e) **quarterly** personnel-file audit results, sanctions reviews, and training-completion rates (per §2.5); (f) **annual** review of the electronic-signature safeguards in §10.7(a), conducted jointly with the IT vendor; and (g) **ad-hoc** immediate reporting of any reportable breach, complaint, licensing visit, or Medicaid audit. The Clinical Director reviews each report, signs acknowledgment, and directs corrective action as needed. The QP retains all compliance reports and Clinical Director acknowledgments in the facility compliance binder for the full record-retention period specified in §1.6.

1.4(b) QP Credentialing Requirements.

The Qualified Professional (QP) for this facility must meet the credentialing requirements of **10A NCAC 27G .0104**. A QP is not required to hold a full, unrestricted clinical license. There are **two acceptable QP pathways** as set out below; either pathway satisfies the QP definition for this facility.

Pathway 1 — Master's Degree plus Recognized NC Credential.

A master's degree in a human services field from an accredited institution, plus **at least one year of full-time, post-master's supervised experience** in the delivery of mental health, developmental disabilities, or substance abuse services to the population served, plus one of the following North Carolina credentials: (i) a **full clinical license** — LCSW, LPC, LMFT, Licensed Psychologist, Licensed Psychological Associate, or psychiatrist (MD/DO); (ii) an **associate or provisional license** — LCSW-A (Associate), LPC-A (Associate), LMFT-A (Associate), or LCAS-P (Provisional); (iii) a recognized **certification** — LCAS (Licensed Clinical Addiction Specialist), CCS (Certified Clinical Supervisor) with a master's degree, or CMSW (Certified Master Social Worker); or (iv) for the nursing discipline, a **Clinical Nurse Specialist (CNS)** or **Nurse Practitioner (NP)** with psychiatric/mental health certification.

Pathway 2 — Bachelor's Degree plus Supervised Experience.

A **bachelor's degree in a human services field** from an accredited institution, plus **two years of full-time, pre- or post-bachelor's supervised experience in the delivery of mental health, developmental disabilities, or substance abuse (MH/DD/SA) services** to the population served. The supervised experience must be documented by the supervising QP and verifiable upon request. A bachelor's-degree-level QP operating under Pathway 2 is not required to hold an NC clinical license, but must complete the NC-DHHS-required QP training modules and must practice under the clinical supervision of a Pathway 1 QP (or the Clinical Director) until such training is completed.

Common Requirements (Both Pathways).

Regardless of pathway, every QP must: (a) complete the **NC-DHHS-required QP training modules** prior to independent practice (or be enrolled and complete them within the probationary period); (b) maintain active credential or supervised-experience documentation as applicable; (c) report any lapse, sanction, restriction, or change in supervised-experience status to the Clinical Director within one business day; and (d) have all verification documents (degree, credential, supervised-experience hours, QP training completion) retained in the personnel file (§2.5). The facility shall verify and document which pathway each QP meets at hire and re-verify annually.

1.5 Data Reporting & Consumer Data Warehouse (CDW).

The facility shall enroll all eligible individuals in the Consumer Data Warehouse (CDW) as required by DMH/DD/SUS. The QP shall ensure termination/discharge from CDW is completed after 60 consecutive days of no billable services. Statistical data required by DHHS, the General Assembly, and federally funded programs shall be submitted per contractual requirements. The QP shall maintain a log of CDW enrollment, updates, and

terminations and review it monthly.

1.6 Record Retention and Disposition.

Clinical service records of minors shall be retained for **12 years after the minor reaches the age of majority (age 18)** — i.e., until age 30 — or longer if required by law or pending litigation. Clinical service records of adults shall be retained for **11 years after the date of the last encounter**, or longer if required. Personnel and administrative records are retained per the DHHS Records Retention and Disposition Schedule. Incident reports and investigatory files are retained per DNCR / Division of Archives and Records requirements.

Destruction of records not listed in an applicable schedule requires prior authorization from DMH/DD/SUS and the Division of Archives and Records via the "Request for Disposal of Unscheduled Records" form. **Record abandonment is strictly prohibited.** If abandonment is suspected, the facility shall notify the payor entity, relevant accrediting organizations, and DHHS, and shall cooperate fully with any investigation. Violations are subject to legal sanctions under HIPAA, 42 CFR Part 2, and the NC General Statutes.

2. Human Resources & Staffing Requirements

Reference: 10A NCAC 27G .0203 & .5600; NC Medicaid CCP 8C; RMDM Chapter 1 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · §2

2.1 Staffing Ratios. Minimum staffing ratios are mandated at all times to ensure resident safety and adequate supervision. Day and evening shifts maintain a 1:4 staff-to-resident ratio (awake and on-site), while the overnight shift maintains a 1:8 ratio (awake — no sleeping permitted). Ratios may be increased based on PCP acuity, behavioral incidents, or 1:1 supervision orders. The QP is responsible for monitoring ratios and adjusting assignments to maintain compliance at all times.

Shift	Minimum Ratio	Status	Notes
Day (7a-3p)	1 : 4	Awake / On-site	May increase per PCP acuity
Evening (3p-11p)	1 : 4	Awake / On-site	May increase per PCP acuity
Overnight (11p-7a)	1 : 8	Awake (No sleeping)	15-minute visual checks required

2.2 Staff Qualifications.

QPs: Meet one of two pathways per §1.4(b) and 10A NCAC 27G .0104 — **Pathway 1:** master's degree in a human services field plus a recognized NC credential (full license, associate/provisional license, certification, or psychiatric nursing credential) plus at least one year of full-time, post-master's supervised MH/DD/SA experience; **OR Pathway 2:** bachelor's degree in a human services field plus two years of full-time, pre- or post-bachelor's supervised MH/DD/SA experience. Both pathways require NC-DHHS QP training modules prior to independent practice.

APs: Bachelor's in human services with at least one year of relevant experience.

Direct Care Professionals (DCPs): High school diploma or GED with at least one year of mental health experience.

2.3 Background Checks (Prior to Unsupervised Contact).

NC SBI fingerprint criminal background check.

Health Care Personnel Registry check.

DSS Child Abuse and Neglect Registry check (every state of residence in prior 5 years).

Motor Vehicle Record (MVR) for staff who transport residents.

Re-checks completed annually and upon reasonable suspicion.

2.4 Mandatory Training (Prior to Independent Duty; Annual Refreshers).

CPR / First Aid (annually)

NCI or CPI restraint and de-escalation (annually)

Medication Administration (RN-delegated, annually)

Bloodborne Pathogens (annually)

Trauma-Informed Care (annually)

Rule 108 incident reporting (annually)

Adolescent development, C-SSRS suicide risk assessment, and person-centered planning (orientation + quarterly)

2.5 Personnel Records.

Personnel files shall contain: position descriptions; education verification; licensure and credentials; continuing education and training records; clinical and administrative supervision documentation; supervision plans; sanctions reviews from professional boards; documented review of the NC Health Care Personnel Registry; and criminal background check results. Records shall be retained per applicable DHHS retention schedules. The QP audits personnel files annually to ensure completeness and compliance with RMDM Chapter 1.

3. Admissions, Discharges, and Transition Planning

Reference: 10A NCAC 27G .5604; NC Medicaid CCP 8C; RMDM Chapters 2 & 5 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · §3

3.1 Admission Criteria. The program serves youth with a primary mental health or behavioral diagnosis requiring supervised living, who are medically stable, and whose clinical needs can be safely met in a Level 3 group home setting. Exclusions include active psychosis requiring Involuntary Commitment (IVC), medical instability, or fire-setting that cannot be safely managed. The QP reviews each referral packet and documents the admission decision.

3.2 Admission Process — Pending vs. Full Records.

Upon initial presentation for screening, a **Pending Record** is created. This contains initial screening information, consultation and administrative coordination, and court-ordered evaluations that do not yet result in active enrollment. If the individual is enrolled in active services, the Pending Record shall be converted into a **Full Clinical Service Record** using the same record number, with all pending documentation merged. On the Day of Admission, the QP obtains signed consents (treatment, medication, transportation, ROI, photographs), the DCP conducts a respectful property search and secures medications, the QP administers the C-SSRS screening and documents immediate clinical status, and an RN visit is scheduled within 72 hours.

3.3 Full Clinical Service Record — Required Elements.

Upon conversion to a Full Record, the chart must contain all applicable elements enumerated in the Comprehensive Clinical Record Content Checklist (Form 8). Categories include: Consents; Demographics and Emergency Information; Health History (including DSM-5-TR/ICD-10 diagnosis and documentation of medication allergies, adverse reactions, and **absence of known allergies**); Medications and Labs; Rights and Restrictive Interventions; Assessments (CCA by a licensed professional); Planning (PCP with MID, service plan, signed service order); Discharge information; Referral information; Service notes/grids; Incidents (filed separately; occurrence noted in record); Disclosures and Legal documents; 42 CFR 2.22 summary for SUD; Accounting of Disclosures; and incoming/outgoing correspondence.

3.4 Discharge & Transition.

Transition planning is initiated at admission and reviewed every 30 days. Discharge may be planned or unplanned. The Discharge Summary is completed within **7 calendar days** of discharge (exceeds RMDM's 30-day requirement), and includes: reason for admission, course/progress, condition at discharge, recommendations, final diagnoses, and dated signatures. Medications are transferred to the guardian with a signed transfer form, belongings are inventoried, and the discharge summary is provided to the guardian, receiving provider, and LME/MCO.

3.5 Administrative Closure.

When a QP or clinician leaves employment without completing required discharge documentation for individuals meeting discharge criteria, the supervisor shall process the discharge. Each administratively closed record shall be audited per entity policy to ensure all billed services are properly documented. If audit reveals unmet documentation requirements, all services billed without proper documentation shall be adjusted back to the payor per 42 CFR 401.305. Documentation of the administrative closure shall be placed in the service record.

4. Clinical Services, Assessments & Person-Centered Planning

Reference: 10A NCAC 27E .0300; NC Medicaid CCP 8C; RMDM Chapter 4 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · §4

4.1 Comprehensive Clinical Assessment (CCA).

A CCA performed by a licensed professional (QP) is required **prior to service delivery**, except in crisis/emergency situations or when a current CCA (with no substantive change) is on file. The CCA must contain: (1) presenting problems, source of distress, precipitating events, associated symptoms; (2) chronological general health, past trauma, behavioral health history (MH/SU including tobacco), treatment history and response; (3) current medications for medical, psychiatric, and SUD treatment, including past ineffective medications or significant side effects; (4) review of biological, psychological, familial, social, developmental, and environmental dimensions identifying strengths, needs, and risks; (5) evidence of beneficiary and LRP participation; (6) analysis and case formulation; (7) **ASAM level of care determination** when substance use disorder is present; (8) DSM-5-TR diagnoses (MH, SUD, IDD, physical health, functional impairment); (9) recommendations for additional assessments, services, supports, or treatment; and (10) dated signature of the licensed professional completing the assessment.

For children and youth, the CCA must address Child and Family Team involvement; if new to services, recommend a Child and Family Team meeting; assess strengths of the child/youth and family (preferably with a strengths-based tool); and utilize IEPs and psychological testing reports if available. **Reassessment** is required when clinically indicated (new behaviors, unmet needs, annual PCP review, or when the service definition requires). If reassessment results in a diagnosis change, a written report is required; if refinement only, a clinical note is sufficient.

4.2 Medical Necessity Determination.

Medical necessity is established by the QP (or licensed/certified professional) based on diagnostic criteria and established best practice guidelines. The CCA and subsequent assessments shall support medical necessity for each service delivered. Medical necessity must meet applicable Medicaid and State-funded policy requirements (NC Medicaid Behavioral Health CCPs; DMH/DD/SUS State-Funded Service Definitions). The QP documents medical necessity in the PCP.

4.3 Person-Centered Plan (PCP) Development.

Within **30 calendar days** of admission, the QP facilitates a PCP meeting including the youth, guardian, LME/MCO representative, and other supports identified by the family. The PCP documents strengths, needs, goals, specific

interventions, and a crisis plan. The PCP is signed by all participants, reviewed every 90 days thereafter, and updated via addendum for significant clinical changes (new diagnosis, medication change, major incident, or living arrangement change). Staff implement PCP goals daily and document progress in shift notes. The QP creates a "Staff Cheat Sheet" summarizing goals, triggers, and interventions for each youth, and staff sign the PCP Acknowledgment Form.

4.4 Service Orders.

Service orders are required for all Medicaid and State-funded services that require them. The service order is indicated by the appropriate professional's signature on the PCP. If a separate format is used, a separate service order is required. **Verbal orders:** treatment may proceed based on a verbal order; the verbal order must be documented in the service record on the date given, specifying the date, who gave the order, who received the order, and each distinct service ordered. Documentation must reflect why a verbal order was obtained in lieu of a written order. The appropriate professional must countersign the order with a dated signature **within 72 hours** of the verbal order.

4.5 Service Authorization.

Requests for authorization are required prior to initiation or continuation of services as per State-funded service definitions, Medicaid CCPs, or the authorizing entity's UM policy. The facility shall notify the authorizing entity when an individual changes providers or ends a service, and end-date reporting requirements must be followed. Service authorizations and reauthorizations are not required to be maintained in the clinical record but shall be available for audit purposes if requested.

5. Behavioral Management & Restraint

Reference: 10A NCAC 27G .0209; CMS Mental Health Parity Rules; RMDM Chapters 2 & 6 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · §5

5.1 Philosophy. Well Spring Intervention LLC employs a trauma-informed, positive behavioral interventions model. We recognize that behavior is communication, and our role is to teach and reinforce safer, more adaptive skills rather than punish behavior. Corporal punishment, withholding meals or sleep, social isolation as punishment, humiliation, and degradation are strictly prohibited and constitute reportable abuse under Rule 108.

5.2 Behavioral Support Plan (BSP).

Developed by the QP within 14 calendar days of admission, based on a Functional Behavioral Assessment (FBA). The BSP includes proactive strategies, replacement skills, de-escalation steps, and crisis responses. All staff review and sign the BSP Acknowledgment Form before working independently with the youth.

5.3 Restraint.

Restraint is an absolute last resort, used only when a youth is in imminent danger of serious harm to self or others and after less restrictive de-escalation has failed. Mechanical, chemical, and seclusion restraints are prohibited. Only NCI/CPI-trained staff may apply restraint, using the least restrictive hold. **Prone (face-down) positions are prohibited.** Post-restraint medical check (within 1 hour), debriefing (within 24 hours), and guardian notification (within 1 hour) are required. All restraints are reported in IRIS.

5.4 Restrictive Intervention Documentation.

Written notifications, consents, approvals, and other documentation per 10A NCAC 27E .0104(e)(9) are required whenever a restrictive intervention is used as a planned intervention. Any planned restrictive interventions must be included in the youth's service plan per 10A NCAC 27E .0104(f). Documentation in the service record must meet requirements of 10A NCAC 27E .0104(g)(2) and (g)(6), including rights restrictions (G.S. §122C-62(e)) and use of

protective devices.

6. Health, Medication, & Nutrition Management

Reference: 10A NCAC 27G .0209; NC Nursing Practice Act; RMDM Chapters 5 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · §6

6.1 Medical Care.

Each resident has an identified Primary Care Physician (PCP) and psychiatrist upon admission. A complete medical examination is conducted within 30 days of admission and annually thereafter. Medical history (including immunizations, allergies, and current prescriptions) is maintained and updated at every visit. **Immunization compliance** for children is documented per NCGS §130A-152. Physician's directions for management of any identified medical conditions must be in the record.

6.2 Tuberculosis (TB) Screening.

Applicable if the facility receives SAPTBG funds for treatment services. Screenings must query: medical treatment in the past 3 months; current residence (jail, streets, shelter, etc.); history of TB tests; and physical symptoms (night sweats, prolonged cough, shortness of breath, unexplained weight loss). Positive responses require referral to the local county health department or medical practitioner for follow-up testing and care. Completed screening and follow-up are documented in the service record.

6.3 Medication Management.

All medications are stored in a double-locked cabinet/cart. Controlled substances are counted and documented at every shift change. Medications are administered only by RN-delegated staff who have completed NC Medication Administration training. Staff follow the "5 Rights" (right youth, right med, right dose, right route, right time) and document administration on the MAR **immediately** (never retrospectively). Medication errors, refusals, and adverse reactions are reported to the RN and QP immediately, documented on the MAR, and entered into IRIS as required.

6.4 Nutrition.

Meals follow USDA guidelines. Special diets are accommodated with provider documentation. Menus are posted and retained 30 days. Food is never withheld as a consequence. Staff preparing food maintain current food handler certifications.

7. Education & Vocational Support

Reference: NC General Statutes (Education of Homeless Children); IDEA; RMDM Chapter 4 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · §7

7.1 Education Coordination.

The QP serves as Education Liaison for every youth in the program. Within 10 calendar days of admission, the QP contacts the Local Education Agency (LEA) — specifically the McKinney-Vento liaison and the EC Director — for enrollment, transportation, and service continuity. The QP signs an ROI for school communication, attends all IEP meetings, advocates for an aligned Behavioral Intervention Plan (BIP), and tracks daily school attendance. School refusal is treated as a clinical issue addressed through the PCP, not as a behavioral violation.

7.2 Vocational Support.

For youth age 14 and older, the PCP includes vocational and independent-living goals aligned with the transition plan. The QP coordinates with Vocational Rehabilitation and community-based employment programs to support

the youth's transition to adulthood and post-discharge stability.

8. Incident Reporting & Response (Rule 108 / IRIS)

Reference: 10A NCAC 27T (Rule 108); RMDM Chapter 6 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · §8

8.1 Reportable Incidents.

The following events are reportable under Rule 108 and must be entered into the Incident Reporting and Investigative System (IRIS): death (including suicide attempts resulting in death), serious injury, alleged abuse or neglect, sexual abuse allegations, use of physical restraint, AWOL/missing child, medication errors, suicidal ideation or self-harm, and any other event meeting the regulatory definition of a reportable incident. When in doubt, staff report.

8.2 Reporting Timeframes.

Staff notify the On-Call QP **immediately** upon discovery of a reportable incident. The QP enters the IRIS report **within 24 hours** of discovery. Allegations of abuse/neglect and missing child trigger **immediate (within 15 minutes)** notification to DSS and law enforcement as appropriate. The guardian is notified **within 1 hour** of incident discovery (unless doing so would compromise an investigation). Verbal notifications are followed by written documentation.

8.3 Investigations.

Internal investigations are initiated within 24 hours and completed within 14 calendar days. Investigations include interviews, video review, documentation review, and a written narrative. A Corrective Action Plan (CAP) is developed for systemic or individual performance issues and uploaded to IRIS.

8.4 Incident Documentation — Separate Filing.

The occurrence of an incident shall be recorded in the service notes. **The completed incident report shall NOT be referenced or filed in the clinical service record.** It shall be filed separately in administrative files. All incident reports are maintained per the Records Retention and Disposition Schedule.

8.5 Abuse/Neglect/Exploitation Reporting.

Relevant facts must be documented in the service record, including reports made by the individual and actions taken. Reporting duties follow G.S. §7B-301 (child abuse), G.S. §108A-102 (disabled adult), and 10A NCAC 27G .0604. The completed incident report is filed separately from the clinical record as required by RMDM Chapter 6.

9. Facility, Safety, & Environmental Management

Reference: 10A NCAC 27G .0600; NC Fire Code; RMDM Chapter 3 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · §9

9.1 Environment.

The facility maintains a safe, clean, home-like environment. Maximum occupancy is two youth per bedroom, with each youth having individual storage space for personal belongings. Indoor temperatures are maintained between 68°F and 80°F. Hot water is regulated to ≤120°F to prevent scalding. Daily safety inspections are conducted by the DCP; weekly inspections by the QP. Hazards are corrected immediately or escalated to the Executive Director.

9.2 Drills & Inspections.

Monthly fire drills target evacuation under 3 minutes. Quarterly tornado drills use the interior safe room. Smoke detectors, fire extinguishers, sprinklers, and CO detectors are inspected monthly (staff) and annually (licensed

contractors). Inspection records are maintained on the Environmental Safety Log (Form 5) for a minimum of 3 years.

9.3 Hazardous Materials.

Cleaning chemicals, sharps, tools, and medications are stored in locked areas. Safety Data Sheets (SDS) are maintained and accessible. No weapons are permitted on facility grounds at any time.

10. Medicaid Billing, Documentation Compliance & Record Management

Reference: NC Medicaid CCP 8C; CMS Documentation Guidelines; RMDM Chapter 6; NCGS Ch. 66 Art. 40 (NC UETA); E-SIGN Act (15 U.S.C. § 7001 et seq.) · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · §10

10.1 Service Notes — General Requirements.

Service notes are the supporting evidence of individual outcomes and must be individualized. **Photocopying or repeating notes verbatim from a prior date or another individual's record is strictly prohibited.** When referencing another individual, use initials, record number, or role (e.g., "sibling") rather than full name unless clinically pertinent and reviewed prior to release. **Timelines:** facility-based services (this program) must be documented at time of service or **within 1 standard business day (not to exceed 24 hours)**. **Late entries** are entries after the required timeframe, must be marked "Late Entry" with the date of entry and the date the service was provided (e.g., "Late Entry made on 7/15/26 for service provided on 7/10/26"), and require a dated signature. Entries exceeding the timeline can only be billed up to **7 business days** beyond the service date; after 7 days, the note is not billable.

10.2 Full Service Note — Required Content.

Every shift/service note must include:

Name of the individual receiving the service (on each page).

Service Record Number issued by the payor along with the **Medicaid Identification Number (MID)** (as applicable), or unique identifier issued by the entity.

Full date the service was provided (month/day/year).

Name of the service provided.

Type of contact: In person, Telehealth, Telephonic, or Collateral.

Place of service.

Purpose of the contact (tied to specific goals in the PCP/ISP).

Description of the interventions, treatment, and support provided (specific and individualized).

Duration: total time spent performing the service (must include active engagement). For this residential per diem service, total time is not required for billing, but interventions must accurately reflect treatment for the event/per diem.

Effectiveness of the intervention(s) and the **individual's response/progress** toward goal(s).

Authentication: electronic signature date stamp and credentials (or handwritten signature with date and credentials). **Use of cursive font in a Word document is NOT a valid electronic signature.** Pre- or post-dating signatures is prohibited.

Additional requirements per Medicaid CCP or State-funded service definition.

Program, Intervention, and Evaluation (PIE) elements are part of the full service note. All required elements must be included for billing and audit purposes. Use Form 7 (Full Service Note Template) as the standardized format.

10.3 Shift Notes (Twenty-Four-Hour Services).

For 24-hour facilities, there must be a note for each shift, and **coverage hours for each shift must be clearly identified** in each note. If no intervention occurs (e.g., youth asleep or at school), the shift note shall reflect the care, oversight, support, and non-treatment events during the shift. When more than one staff person is providing services for a shift, **only one staff is required to document and sign the shift note**; however, the note must **identify the names and titles/positions of other staff persons present** to demonstrate staffing ratios are met.

10.4 Modified Service Notes & Grids.

Modified notes may be used only for services where explicitly allowed in CCP/State Service Definitions (e.g., Respite). Modified notes must include: Name, Service Record #/MID/Unique ID, Service provided, Date, Duration, Goal addressed/tasks performed, and Full dated signature/credentials (or initials if full signature is on the page).

Grids may be used only for specified services (e.g., Residential Supports — NC Innovations, Respite). Grids must include: Name, Service Record #/MID/Unique ID, Service, Date, Duration, Goals/Interventions, and Full dated signature/credentials (or initials if full signature is on the page). For NC Innovations, grids must include an intervention key, progress key, and a comment section with dated entries. **This facility will primarily use full shift notes.** If using grids for Respite, the grid must meet all requirements above.

10.5 Alterations to Service Documentation.

The original entry must not be deleted or altered so it is not clearly legible in its original context. Corrections must be made by the person who initially authored the entry, with an explanation of the correction. If that is not possible, the person updating the documentation must reflect the needed change and the rationale. Any change must be **signed and dated**. The revised/corrected entry must be in proximity to the original entry. EHR systems with tracked changes and timestamps satisfy these guidelines. **Alterations exceeding 7 business days beyond the service date are not billable.**

10.6 Authenticating Service Documentation.

All entries are validated via signature (EHR or paper) with signature date, credentials, degree, licensure, and/or title.

Initials are only permitted for updates/corrections or for notations entered on a service grid (where full signature is noted on the grid signifying appropriate initials). **Pre- or post-dating signatures in any form is prohibited.**

ADA Accommodations: if an individual has a documented medical/physical reason (per ADA) for not being able to sign, another means for providing signature is acceptable; the process must be incorporated in facility policy.

Unavailable Original Author: if the original author is no longer available to sign or amend an entry, a notation reflecting this shall be documented in the record and signed/dated by the appropriate party. **Rubber Stamps:** only used for medical/physical reasons with ADA accommodations; if unable to use stamp, the individual may authorize someone of their choosing in writing.

10.7 Electronic Signatures.

Electronic signatures are permitted for authenticating service documentation, consents, treatment plans, and other records governed by this manual, in accordance with the **North Carolina Uniform Electronic Transactions Act (NCGS Chapter 66, Article 40)** and the federal E-SIGN Act (15 U.S.C. § 7001 et seq.). An electronic signature is an electronic sound, symbol, or process attached to or logically associated with a record and executed or adopted by a person with the intent to sign the record. Where electronic signatures are used, they carry the same legal weight and enforceability as handwritten signatures for transactions conducted electronically. **Use of a cursive font in a word-processing document does not constitute a valid electronic signature**; the signature must be applied through the facility's authenticated EHR/EMR system or an approved secure signing platform that captures the signer's identity, timestamp, and intent. Each electronic signature shall be uniquely linked to the signer, render any subsequent modification detectable, and include an auditable timestamp reflecting the date and time of execution.

10.7(a) Safeguards.

The facility shall implement and maintain reasonable administrative, technical, and physical safeguards to ensure the integrity, confidentiality, and non-repudiation of electronic signatures, consistent with the HIPAA Security Rule (45 CFR Part 164 Subpart C), 42 CFR Part 2, and NCGS § 66-40(d). Required safeguards include: (a) **unique user credentials** for each authorized signer, with shared logins strictly prohibited; (b) **multi-factor authentication** where technically feasible; (c) **automatic session timeout** after no more than 15 minutes of inactivity; (d) **encrypted transmission** (TLS 1.2 or higher) and **at-rest encryption** of all signed records; (e) **immutable audit trails** capturing the signer's identity, date, time, IP address or device identifier, and the document version signed; (f) **role-based access controls** limiting signature authority to staff with appropriate licensure, credentials, and documented training; (g) **immediate revocation** of credentials upon termination, role change, or suspected compromise; and (h) **annual review** by the QP and IT vendor to verify continued compliance. Lost, stolen, or shared credentials must be reported to the QP within one business day; the facility shall deactivate the compromised credentials and investigate the scope of any unauthorized signatures before reissuing access.

10.7(b) System Unavailability Procedures.

In the event the EHR/EMR system or approved electronic signing platform is unavailable — whether due to planned maintenance, internet outage, vendor outage, cyber-incident, or natural disaster — staff shall revert to paper-based documentation and handwritten signatures with date, credentials, and title. The QP or designee shall declare a **Documentation Continuity Event**, notify all shifts in writing (or by phone tree if email is unavailable), and distribute blank copies of the required forms (Shift Note, MAR, Restraint & Debriefing Checklist, Incident Report). Staff shall legibly sign and date each paper entry in blue or black ink. When the electronic system is restored — and in no event later than **72 hours** after the original entry — the originating staff member (or, if unavailable, the on-duty QP) shall transcribe the paper entry verbatim into the electronic record, mark the entry as "*Late Entry — System Unavailability (date/time of restoration)*", reference the original paper document by scan or filename, and electronically sign the transcribed entry. The original paper document shall be scanned and attached to the electronic record, then retained as a source document per the record retention policy (§1.6) for the full retention period. Paper entries that cannot be transcribed within **7 business days** must be reviewed by the QP, who shall document the reason for delay and the corrective action taken. The QP shall maintain a log of all Documentation Continuity Events, including the start/end time, cause, records affected, and verification that all paper entries were transcribed and electronically authenticated.

10.8 Service Authorizations & End-Date Reporting.

The facility shall maintain service authorizations/reauthorizations in a separate audit file (not required in clinical record but available upon request). When a youth changes providers or ends a service, the facility must notify the authorizing entity per their specified protocol. End-date reporting shall follow the authorizing entity's requirements.

10.9 Billing.

The program bills Medicaid on a daily per diem basis. Billing is suspended for any day the youth is hospitalized, detained, or on a home pass exceeding **24 consecutive hours**. The Home Pass & Medicaid Billing Exclusion Tracker (Form 4) documents all absences and corresponding billing action. Billing errors are corrected promptly and reported to the LME/MCO as required. Records are released only with a signed ROI or as required by law, court order, or regulatory audit.

11. Privacy, Security, Confidentiality & Access to Records

Reference: RMDM Chapter 3; HIPAA Privacy/Security Rule; 42 CFR Part 2; HITECH Act; NCGS §122C-52 through 122C-56 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · §11

11.1 Privacy and Security Policies.

The facility shall maintain comprehensive policies addressing how information is recorded, stored, retrieved, disseminated, and protected against loss, theft, destruction, unauthorized access (breach), and natural disasters. Compliance with the HIPAA Privacy and Security Rules, the Omnibus HIPAA final rule, the HITECH Act, and ARRA is mandatory. Policies shall address electronic communication (email, voice messaging, SMS/text speak, EHR usage). Where multiple requirements exist, the **most stringent** applies, including professional codes of ethics.

11.2 Safeguards.

Formal policies and procedures shall reasonably protect against unauthorized uses and disclosures of PHI, and against reasonably anticipated threats or hazards (42 CFR § 2.16). Secure storage: locked, climate-controlled areas with limited access; electronic records backed up daily and stored securely off-site.

11.3 Confidentiality.

Compliance is required with NCGS §122C-51 through 122C-56; 10A NCAC 26B; **42 CFR Part 2** (Confidentiality of Alcohol and Drug Abuse Patient Records); and NCGS §130A-143 (AIDS and related conditions). **Substance Use Information:** requires the individual's written authorization before disclosure to other treatment providers (unless court order or allowable exception per 42 CFR Part 2). Absent written consent, entities must redact all identifying information from alcohol/drug records when sharing. **Care Coordination Exceptions:** under HIPAA and NCGS §122C-55(a), entities may share confidential information for coordination of care/treatment without written consent (requirements of NCGS §122C-55(a)7 apply); however, 42 CFR Part 2 still requires written authorization for SUD records.

11.4 Disclosure Documentation and Accounting.

Authorization must be in writing, voluntary, and informed (understanding what is exchanged, with whom, and for what purpose). Staff must verify the identity of the person requesting PHI and obtain required documentation/representations per Omnibus HIPAA rules. The **minimum necessary** standard (164.502(b)) applies. **Accounting of Disclosures** must be maintained for a minimum of **six years**. The accounting must include: name of the individual; medical record or ID number; date the information was released/disclosed; provider/entity/agency/individual to whom information was released; purpose of the release/disclosure; description of specific information released/disclosed; and name of person disclosing the information (recommended). All disclosures or re-disclosures of SUD treatment information must comply with 42 CFR Part 2; re-disclosure is prohibited except per NCGS §122C-53 through 122C-56. Releases/disclosures from external entities must be contained in the individual's service record.

11.5 Individual Access to Service Records.

Individuals and Legally Responsible Persons (LRPs) have the right to access information contained in their service record, per NCGS and DHHS provisions. **Limited Access:** if access is restricted, proper justification for restricting access must be clearly indicated in the service record. Individuals have the right to appeal such determination. Refer to Article 3, Part 1 (Client Rights), the Omnibus HIPAA Final Rule, and DHHS Privacy/Security policies.

11.6 Transporting Records.

Service records shall only be transported by designated individuals (hard copy, USB, tablet, CPU, etc.). When removed from premises, records shall be secured in a locked compartment and protected from unauthorized access. Policy must address what occurs if information is lost or stolen during transport, including immediate notification of the QP, Compliance Officer, and — if PHI is compromised — notification of affected individuals and DHHS per breach notification rules.

11.7 Storage and Maintenance.

Storage and maintenance are consistent with privacy/security principles. Electronic records self-warranty per NC State Archives guidelines. "Managing Public Records Produced by Information Technology Systems" guidelines apply. The QP conducts quarterly audits of storage security and access logs.

PART 2

Actionable Workflows & Protocols

These 22 protocols translate the SOP policies into clear, sequential, step-by-step actions for daily operations. Each protocol is designed to be referenced quickly during a shift and followed exactly. Deviations from protocol require QP approval and must be documented. Protocols 20 and 21 are new in Version 2.0 and address service orders/authorizations and record management/disclosure accounting per RMDM requirements. Protocol 22 is new in Version 2.9 and codifies the daily workflow schedules for every personnel classification so that each shift, role, and hand-off has a documented time-blocked routine.

1. Admissions & Pending Record Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 1: Admissions & Pending Record Protocol

Pre-Admission. QP reviews referral packet, verifies active Medicaid, schedules tour with guardian and youth.

Pending Record Creation. If individual presents for screening but is not immediately enrolled, create a Pending Record with screening/consultation info.

Day of Admission. QP obtains all signed consents (treatment, medication, transportation, ROI, photos). DCP conducts respectful property search and secures medications. QP administers C-SSRS and documents clinical status. Convert Pending Record to Full Clinical Record (merge all documentation).

Post-Admission. RN visits within 72 hours for medication delegation and nursing assessment. ISP/PCP meeting scheduled within 30 days.

2. Daily Direct Care Protocol (DCP)

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 2: Daily Direct Care Protocol (DCP)

Day Shift (7a-3p). Shift change handoff and controlled-substance med count. Wake youth per schedule with 15-minute visual checks. Breakfast and AM medications. School transportation. Daily chores.

Evening Shift (3p-11p). Shift change. Therapy and clinical groups. Chores, dinner, and PM medications. Free time and hygiene routine. 10:00 PM bedtime.

Night Shift (11p-7a). Shift change and med count. Conduct 15-minute line-of-sight room checks documented on the Night Watch Log. Awake at all times — no sleeping permitted.

3. Incident Response & IRIS Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 3: Incident Response & IRIS Protocol

Immediate. Ensure scene safety; call 911 if medical emergency or active danger. Notify the On-Call QP immediately.

Notifications. DSS immediately for abuse allegations. Guardian within 1 hour. LME/MCO per their requirements.

Reporting. QP enters the IRIS report within 24 hours using objective, factual narrative — no interpretations or labels.

Investigation. QP completes the internal investigation within 14 days and uploads findings and CAP to IRIS.

Separate Filing. Record the occurrence of the incident in the service note. File the completed incident report separately in administrative files — NOT in the clinical record.

4. Behavioral Crisis & Restraint Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 4: Behavioral Crisis & Restraint Protocol

Tier 1 — De-escalation. Use BSP coping skills: sensory items, breaks, verbal redirection, change of environment.

Tier 2 — Crisis. Clear the area of other youth. Call for staff backup. Use the NCI/CPI verbal continuum and physical distancing.

Tier 3 — Restraint. Only for imminent serious harm. Apply NCI hold; never use prone, mechanical, or chemical restraints. End the hold the moment danger passes.

Post-Restraint. Medical check within 1 hour. Notify guardian within 1 hour. Debrief with youth within 24 hours. File IRIS report.

5. Medication Administration Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 5: Medication Administration Protocol

Prep. Wash hands. Unlock med cart. Verify the "5 Rights" against the MAR: right youth, right med, right dose, right route, right time.

Administer. Verify youth identity. Pour medication into a labeled cup. Observe swallowing. Check the mouth for "cheeking".

Document. Initial the MAR immediately. Circle "R" if refused; notify RN and QP. Never pre-initial or backdate.

Errors. Stop, isolate remaining meds, call RN/Psychiatrist, monitor the youth, file IRIS, and notify the guardian.

6. Medicaid Service Note & Documentation Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 6: Medicaid Service Note & Documentation Protocol

Write the shift note by end of shift. No backdating — ever.

Mandatory Fields. Youth Name & Service Record # / MID. Full Date & Place of Service. Name of Service & Type of Contact (in-person, collateral, etc.). Purpose (ISP Goal # referenced). Objective Description of Interventions (factual, specific). Duration (for time-based services; for per diem, indicate shift coverage hours). Effectiveness & Youth Response/Progress. Full Signature with Credentials and Date.

Shift Notes. Identify coverage hours and names of all staff present for ratios.

Late Entries. Mark "Late Entry made on [date] for service on [date]" if after 24 hours.

Alterations. Correct with explanation, signed/dated; never obscure original. Alterations exceeding 7 business days are not billable.

Billing suspended for any day youth is away for more than 24 consecutive hours.

7. AWOL / Missing Child Response

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 7: AWOL / Missing Child Response

Discovery. Call the staff code for missing youth. Search the immediate area and check all cameras.

Notifications (within 15-30 minutes). Law enforcement, guardian, LME/MCO, and DSS (if applicable).

Documentation. File IRIS within 24 hours with a complete narrative.

Return. Conduct a medical check, contraband search, and trauma-informed debriefing. Notify all parties who were previously contacted.

8. Suicide Risk & Self-Harm Management

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 8: Suicide Risk & Self-Harm Management

Screening. C-SSRS at admission and immediately whenever threats or self-harm occur.

Safety. Remove hazards (ligatures, sharps, cords). Implement 1:1 line-of-sight supervision.

Clinical. QP assesses within 1 hour. If imminent risk: call 911 or initiate IVC. If moderate risk: update the Safety Plan and increase supervision.

Documentation. File IRIS. Staff sign the updated Safety Plan and acknowledge the increased supervision level.

9. Contraband & Room Search Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 9: Contraband & Room Search Protocol

Definition. Contraband includes weapons, drugs, unauthorized medications, lighters, chargers with frayed cords, and any item prohibited by house rules.

Routine. Conducted at admission and on return from any pass. The youth must be present.

Probable Cause. Requires QP approval. Two staff must be present. Document the basis for the search.

Execution. Search systematically (one area at a time). Safe items are locked in the safe and returned at discharge. Illegal items are turned over to law enforcement.

10. PCP Implementation & Review

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 10: PCP Implementation & Review

Development (30 Days). QP schedules the meeting with the team and drafts measurable, individualized goals.

Operationalize. QP creates a "Staff Cheat Sheet" summarizing each youth's goals, triggers, and interventions. Staff sign the PCP Acknowledgment Form.

90-Day Review. QP tracks expirations and schedules reviews. Updates progress, obtains signatures, and writes addendums for any significant change.

Service Order. The signed PCP serves as the service order for all services delivered.

11. Transportation & Community Outing Safety

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 11: Transportation & Community Outing Safety

Driver Prep. Valid license, clean MVR on file. Complete the daily vehicle safety checklist. Confirm first-aid kit and Medicaid cards are present.

Transport. Conduct a headcount before leaving and upon return. If transporting one youth, staff sits in the back seat. Seatbelts on at all times. Never leave youth unattended in a vehicle.

Outings. Tied to an ISP goal. Assess behavioral stability (no restraint in the last 4 hours). Carry the agency cell phone and emergency contact list.

12. Grievance Process for Youth

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 12: Grievance Process for Youth

Accessibility. Process is explained at admission. Blank forms and a locked grievance box are located in the common area. LME/MCO grievance numbers are posted.

Filing. Verbal or written. Staff assist immediately upon request. Place the grievance in the locked box within 1 hour.

Resolution. QP opens the box daily. Acknowledges within 24 hours. Provides written resolution within 7 days. Abuse allegations trigger an IRIS report and immediate DSS notification.

13. Staffing Ratio & Awake Overnight Protocols

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 13: Staffing Ratio & Awake Overnight Protocols

Ratios. 1:4 day and evening. 1:8 overnight. If a staff member calls out, the on-call system activates a replacement; the QP covers in-house if no replacement is available.

Awake Overnight. Sleeping is prohibited. 15-minute visual room checks are documented on the Night Watch Log. Hallway lights remain on. Engage nighttime wakers quietly and briefly — do not start conversations that escalate arousal.

14. Medical Emergencies & Acute Illness Response

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 14: Medical Emergencies & Acute Illness Response

Triage. Assess the situation. Call QP and/or RN. Err on the side of caution — when in doubt, transport.

Emergency. Call 911. Staff rides in the ambulance. QP calls in additional staff to maintain ratio at the facility.

Non-Emergency. Contact PCP for a same-day appointment. Take consent forms and Medicaid card.

Documentation. Guardian notified within 1 hour. IRIS filed. Billing suspended if the youth is hospitalized overnight.

15. Educational Coordination & School Reintegration

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 15: Educational Coordination & School Reintegration

Enrollment (5 Days). QP contacts the LEA McKinney-Vento liaison or EC Director. Signs ROI. Coordinates transportation.

IEP. QP/AP attends IEP meetings. Advocates for an aligned BIP. Files the IEP in the youth's chart.

Attendance. Morning check before transport. Log daily. Refusal is an ISP issue, not a punishable offense. Collect daily behavior reports from the school.

16. Family Engagement, Visitation & Home Passes

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 16: Family Engagement, Visitation & Home Passes

Visitation. Based on ISP/court orders. Approved visitor list is maintained in the office. Staff search bags before visits. Document all visits.

Home Passes. QP and guardian must approve. Pass medications are provided in a locked box with administration instructions.

Billing Exclusion. If the youth is away for more than 24 consecutive hours, Medicaid per diem is suspended. Notify the Billing Coordinator immediately.

17. Discharge & Transition Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 17: Discharge & Transition Protocol

Planning. Begins at admission. Trial home visits are conducted prior to discharge whenever possible.

Step-Down. Discharge meeting is scheduled. Community linkages (outpatient therapy, medication management, school) are established and confirmed before discharge.

Discharge Day. Medications are handed directly to the guardian with a signed transfer form. Belongings are inventoried. Discharge summary is completed within 7 days.

Administrative Closure. If QP/clinician leaves without completing discharge documentation, the supervisor processes the discharge and audits the record. Billed services without proper documentation are adjusted back to the payor per 42 CFR 401.305.

18. Infectious Disease & Bloodborne Pathogen Control

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 18: Infectious Disease & Bloodborne Pathogen Control

Universal Precautions. Treat all bodily fluids as infectious. Wear appropriate PPE (gloves, mask, eye protection) when exposure is possible.

Exposure. Wash the affected area with soap and water. Notify QP and RN. Go to Urgent Care within 2 hours. File IRIS. Offer post-exposure testing per protocol.

Outbreak. Two or more youth with identical symptoms = isolate, increase sanitation, and notify the County Health Department if the illness is reportable.

19. Emergency & Disaster Preparedness

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 19: Emergency & Disaster Preparedness

Fire. Evacuate immediately. Headcount at the rally point. Call 911. Do not re-enter. Monthly drills required.

Tornado. Move to the interior safe room. Turtle position. Quarterly drills required.

System Failure. Distribute flashlights. If heat loss exceeds 4 hours or water loss exceeds 8 hours, initiate the Emergency Relocation Plan.

Lockdown. Secure doors and windows. Hide. Silence phones. Do not open for anyone but law enforcement. Annual drills required.

20. Service Orders & Authorization Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 20: Service Orders & Authorization Protocol

Service Order. The PCP shall serve as the service order when signed by the appropriate professional. If a separate format is used, ensure a separate service order is signed.

Verbal Orders. If a verbal order is obtained, document in the record on the date given: date, who gave, who received, services ordered, and reason for verbal. The professional must countersign within 72 hours.

Authorization. Submit authorization requests to the authorizing entity prior to initiation or continuation per their UM policy.

End-Date Reporting. Notify authorizing entity immediately when youth changes providers or ends service. Follow entity-specified protocol.

Maintain authorization/reauthorization records in an audit-ready file (not in clinical record).

21. Record Management, Retention, Access & Disclosure Accounting

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 21: Record Management, Retention, Access & Disclosure Accounting

Retention. Minors — 12 years after age 18 (until age 30); Adults — 11 years after last encounter.

Personnel/Admin records per DHHS schedule. Records not on a schedule require DMH/DD/SUS authorization before destruction.

Record Abandonment. Strictly prohibited. Notify payor, accreditor, and DHHS if suspected. Violations are subject to legal sanctions.

Access. Youth/LRP may access records. If access is restricted, document justification in record. Appeal rights apply.

Accounting of Disclosures. Maintain Form 9 (Accounting of Disclosures Log) for each youth. Include: Name, Record #, Date disclosed, Recipient, Purpose, Description of info, Disclosing party. Retain for minimum 6 years.

Confidentiality. Comply with 42 CFR Part 2 for SUD records. Obtain written ROI for SUD disclosures. Minimum necessary standard applies.

Transport. Only designated staff; secured in locked compartment; documented policy for loss/theft.

22. Daily Workflow Schedules for All Personnel

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 2 · Protocol 22: Daily Workflow Schedules for All Personnel

This protocol codifies the daily workflow schedule for every personnel classification at Well Spring Intervention LLC. Each role has a time-blocked routine that aligns with the 24/7 residential operation, the 1:4 day/evening and 1:8 overnight staffing ratios required by 10A NCAC 27G .5600, and the documentation cadences required by the RMDM and Rule 108. Schedules are templates — actual shift assignments may flex to cover call-outs, school transportation, medical appointments, and clinical visits, but every role must complete its documentation obligations by end of shift. The QP maintains the master staffing schedule; deviations are tracked on the Shift Change & Awake Night Watch Log (Form 1).

(a) Qualified Professional (QP)

The QP is the clinical and operational lead on-site. The QP does not carry a fixed 1:4 youth-supervision caseload; instead, the QP floats across the program to deliver clinical services, supervise staff, and ensure documentation compliance. The QP reports to the Clinical Director per §1.4 and §1.4(a).

Time	Activity
7:00 AM	Arrive; review the prior 24-hour Night Watch Log (Form 1) and IRIS queue. Brief with off-going overnight staff. Verify controlled-substance count.
7:30 AM	Lead shift-change huddle with on-coming DCPs. Assign youth supervision ratios. Review Behavior Support Plans and any overnight incidents.
8:00 AM	Wake youth per house schedule. Coordinate breakfast and AM medications with the DCP. Confirm school transportation.
9:00 AM	Begin clinical block: individual therapy sessions, PCP/ISP reviews, family collateral calls, or coordination with the LEA / IEP team.
11:00 AM	Audit prior day's service notes for RMDM compliance (Form 7 template). Flag any backdated, missing, or non-compliant notes for correction before the 24-hour deadline.
12:00 PM	Lunch (youth supervised by DCP). QP reviews email, prior-authorization requests, and any LME/MCO correspondence.
1:00 PM	Clinical block continued: CCA assessments, ASAM screenings (if SUD), safety-plan updates, or scheduled psychiatric telehealth visits.
3:00 PM	Shift-change huddle with on-coming evening DCPs. Brief on any clinical concerns, restrictions, or supervision-level changes.
4:00 PM	Group therapy or skill-building group (QP leads or co-leads with an AP). Document each youth's response in a service note.
5:30 PM	Family visitation oversight; review approved visitor list; coordinate bag searches with the DCP.
7:00 PM	Documentation block: complete all clinical service notes for the day, sign PCP addendums, file Form 9 disclosure entries.
8:00 PM	Final round — verify evening medications administered and MAR initialled. Confirm tomorrow's appointments and transportation.
9:00 PM	Shift-change huddle with overnight DCP. Hand off clinical concerns, supervision-level changes, and any pending IRIS investigations.
9:30 PM	Depart. On-call QP coverage begins; carry the agency phone until 7:00 AM next day.

(b) Associate Professional (AP) / Paraprofessional (PP)

APs and PPs provide direct behavioral-health services under QP supervision per §2.2. They carry a 1:4 youth-supervision ratio on day or evening shift and lead structured clinical and skill-building activities. APs may co-lead group therapy with the QP.

Time	Activity
7:00 AM	Arrive; controlled-substance count with off-going overnight DCP. Review the Night Watch Log and any incident reports from overnight.
7:30 AM	Shift-change huddle with QP. Receive youth-supervision assignment and BSP talking points for the day.
8:00 AM	Wake youth; assist with morning hygiene and AM routine. Administer AM medications per MAR (under RN delegation).

Time	Activity
8:30 AM	Breakfast; supervise mealtime behaviors and document any concerns. Coordinate school transportation with the DCP.
9:30 AM	Structured clinical block: lead or co-lead a skill-building group (coping skills, emotional regulation, social skills). Document each youth's participation.
11:00 AM	Individual check-ins with assigned youth (15 min each). Review BSP goals, address any overnight stressors, update the youth's daily log.
12:30 PM	Lunch supervision; chore rotation; structured free time.
2:00 PM	Co-lead group therapy with QP (or lead an AP-run activity group). Document interventions and youth response.
3:00 PM	Shift-change huddle with evening DCP. Hand off youth-supervision assignment, behavior status, and any clinical concerns.
3:30 PM	Continue with youth supervision. Coordinate homework support, recreational activities, and community outings per ISP goals.
5:00 PM	Family visitation support (if scheduled). Coordinate bag searches with the DCP; supervise visitation room.
6:00 PM	Dinner; supervise mealtime. Administer PM medications per MAR.
7:30 PM	Wind-down routine: hygiene, structured quiet time, journaling, or sensory activities per BSP.
9:00 PM	Youth bedtime routine. Document the day's service note (Form 7) before end of shift.
10:00 PM	Shift-change huddle with overnight DCP. Brief on each youth's emotional status, sleep considerations, and any restrictions.
11:00 PM	End of shift. On-call AP coverage rotates weekly.

(c) Direct Care Professional (DCP) — Day Shift (7a-3p)

DCPs carry the primary 1:4 youth-supervision ratio. Day-shift DCPs focus on morning routine, school transportation, and structured daytime activities.

Time	Activity
6:45 AM	Arrive; clock in. Receive shift-change briefing from overnight DCP. Controlled-substance count verified and signed.
7:00 AM	Confirm youth headcount; verify all youth are awake and accounted for. Begin AM room inspections.
7:30 AM	Shift-change huddle with QP and AP. Receive youth-supervision assignment and BSP talking points.
8:00 AM	Wake any youth still sleeping; assist with morning hygiene. Administer AM medications per MAR.
8:30 AM	Breakfast; supervise mealtime. Verify each youth eats; document any food refusal or concerns.
9:00 AM	School transportation: headcount before leaving and upon arrival. Confirm seatbelts. Drop off at school; obtain school behavior report.
10:00 AM	Return to facility. House chores: assign and supervise cleaning rotations per the chore chart. Document completion.
11:00 AM	Structured activity block: outdoor time, life-skills lesson, or community outing per ISP goals. Maintain line-of-sight supervision.
12:30 PM	Lunch; supervise mealtime. Administer midday medications if ordered.
1:30 PM	Documentation block: complete the daily log for each assigned youth. Note any behavioral incidents, BSP interventions used, and effectiveness.

Time	Activity
2:00 PM	School pickup: headcount; obtain daily behavior report from school staff. Transport back to facility.
2:45 PM	Shift-change preparation: complete handoff notes for evening DCP. Verify controlled-substance count.
3:00 PM	Shift-change huddle with evening DCP. Brief on each youth's day, school reports, and any incidents. End of shift.

(d) Direct Care Professional (DCP) — Evening Shift (3p-11p)

Evening-shift DCPs cover the after-school and bedtime routine. They administer PM medications, supervise dinner and evening activities, and prepare youth for sleep.

Time	Activity
2:45 PM	Arrive; clock in. Review the prior shift's handoff notes and the Night Watch Log.
3:00 PM	Shift-change huddle with day DCP and QP. Receive youth-supervision assignment, school behavior reports, and any restrictions.
3:30 PM	Controlled-substance count verified and signed. Receive keys, agency phone, and any visitor approvals.
4:00 PM	After-school snack; supervised homework time. Coordinate with AP on any scheduled therapy sessions.
5:00 PM	Family visitation (if scheduled). Conduct bag searches; supervise visitation; document the visit in each youth's log.
6:00 PM	Dinner preparation and mealtime. Administer PM medications per MAR; verify swallowing; check mouth for "cheeking".
7:00 PM	Evening activity: structured recreation, life-skills lesson, or community outing per ISP goals.
8:30 PM	Wind-down routine: hygiene, pajamas, quiet activities (reading, journaling, sensory items per BSP).
9:30 PM	Youth bedtime. Confirm each youth is in their assigned bed. Begin 15-minute room checks (continue into overnight shift).
10:00 PM	Documentation block: complete the daily log and shift note (Form 7) for each assigned youth. Address any behavioral incidents.
10:45 PM	Shift-change preparation: complete handoff notes for overnight DCP. Verify controlled-substance count.
11:00 PM	Shift-change huddle with overnight DCP. Brief on each youth's evening, bedtime status, and any concerns. End of shift.

(e) Direct Care Professional (DCP) — Awake Overnight Shift (11p-7a)

Awake overnight DCPs maintain 1:8 line-of-sight supervision. Sleeping is strictly prohibited. 15-minute visual room checks are documented on the Night Watch Log (Form 1) throughout the shift.

Time	Activity
10:45 PM	Arrive; clock in. Review the prior shift's handoff notes. Receive keys and agency phone.
11:00 PM	Shift-change huddle with evening DCP. Receive youth-supervision assignment and any nighttime considerations (sleepwalking, night terrors, etc.).
11:15 PM	Controlled-substance count verified and signed. Confirm hallway lights remain on. Verify all youth are in their assigned beds.
11:30 PM	Begin 15-minute room checks (continue throughout the shift). Each check: visually confirm each youth breathing and in bed; initial the Night Watch Log (Form 1).

Time	Activity
12:00 AM	House security check: verify all exterior doors and windows locked. Verify alarm system armed. Document on the Night Watch Log.
1:00 AM	Administrative block: restock supplies, sanitize common areas, prepare for morning routine (set out breakfast supplies, prep medications for AM count).
3:00 AM	Continue 15-minute room checks. Engage nighttime wakers quietly and briefly — do not start conversations that escalate arousal. Document any incidents.
5:00 AM	Begin AM preparations: start coffee, set the table, prepare breakfast menu. Confirm AM medication pull for the day DCP.
6:00 AM	Final security check. Complete end-of-shift Night Watch Log summary. Document any incidents, AWOL attempts, or medical concerns from overnight.
6:45 AM	Day-shift DCP arrives. Controlled-substance count verified and signed. Brief on overnight events.
7:00 AM	Shift-change huddle with day DCP and QP. Hand off any concerns. End of shift.

(f) House Manager

The House Manager is a senior DCP who supervises day-to-day facility operations, inventory, maintenance, and the chore system. The House Manager reports to the QP and works Monday-Friday 9a-5p with on-call availability.

Time	Activity
9:00 AM	Arrive; review the prior 24 hours of Night Watch Logs and incident reports. Brief with the QP on any facility issues.
9:30 AM	Facility walk-through: inspect all common areas, bedrooms, bathrooms, kitchen, and outdoor space. Document any maintenance needs.
10:00 AM	Inventory check: food, cleaning supplies, PPE, first-aid supplies. Place orders as needed within the household budget.
10:30 AM	Coordinate with maintenance vendors for any scheduled repairs or inspections. Document all work orders.
11:30 AM	Review and update the chore chart. Meet briefly with each youth to assign/reinforce chore expectations.
12:30 PM	Lunch.
1:00 PM	Audit the medication cart with the QP and RN (if on-site). Verify MAR completion, controlled-substance log, and expired medication disposal.
2:00 PM	Compliance audit: check fire-extinguisher tags, smoke-detector test logs, environmental safety logs (Form 5). Schedule any missing drills.
3:00 PM	Brief with the on-coming evening DCP. Reinforce any facility-related instructions for the shift.
4:00 PM	Administrative block: file maintenance records, update the household budget tracker, prepare the weekly facility report for the QP.
5:00 PM	End of shift. On-call availability for facility emergencies (water leak, heating failure, alarm activation) until 9:00 AM next day.

(g) Registered Nurse (RN)

The RN provides medical oversight per §6 and 10A NCAC 27G .5600. The RN visits the facility at minimum weekly and within 72 hours of any new admission, and is on-call 24/7 for medical questions and medication errors.

Time	Activity
9:00 AM	Arrive on scheduled visit day. Brief with the QP on any medical concerns, medication changes, or recent incidents.
9:30 AM	Medication cart audit: verify MAR completion, inspect for expired medications, review any PRN administration patterns.
10:00 AM	Individual youth health checks: vital signs if ordered, weight checks, skin checks, assessment of any reported symptoms.
11:00 AM	Coordinate with the prescribing psychiatrist via telehealth or phone. Document any medication changes and update the MAR.
12:00 PM	Train DCPs on any new medication orders, administration techniques, or delegation updates per 10A NCAC 27G .5600.
1:00 PM	Documentation block: complete nursing notes in each youth's chart. Update the medication administration record. File any new lab orders.
2:00 PM	Coordinate medical appointments: schedule PCP visits, dental visits, vision screenings, and any specialty referrals.
3:00 PM	Brief with the QP and House Manager on any medical-action items. Depart facility; on-call coverage continues 24/7.
On-Call	Available by phone for medication errors, adverse reactions, acute illness triage, and any medical questions. Responds within 15 minutes; documents all calls in the nursing log.

(h) Billing Coordinator

The Billing Coordinator manages Medicaid per-diem billing, authorization tracking, and suspension-day accounting. The Billing Coordinator reports to the Executive Director and works Monday-Friday 8a-4p remotely or on-site.

Time	Activity
8:00 AM	Arrive; review the prior day's admission/discharge log. Confirm Medicaid eligibility for each youth via NCTracks.
8:30 AM	Process the prior day's per-diem billing. Verify each youth was in residence; apply any suspension-day exclusions from Form 4.
9:30 AM	Track authorization end dates. Submit reauthorization requests to the LME/MCO at least 14 days before expiration.
10:30 AM	Reconcile any claim denials. Document the reason for each denial and the corrective action taken. Escalate systemic issues to the QP.
12:00 PM	Lunch.
1:00 PM	Coordinate with the QP on any youth who were AWOL, hospitalized, or on home pass in the prior 24 hours. Apply billing exclusions per Protocol 16 and Form 4.
2:00 PM	Audit service-note completion in the EHR. Flag any youth with missing notes from the prior 24 hours; notify the QP for correction before the 7-business-day alteration deadline.
3:00 PM	Prepare the weekly billing summary for the Executive Director. Include total billable days, suspension days, denials, and pending authorizations.
4:00 PM	End of shift. On-call availability for billing-related questions during business hours; the QP handles after-hours billing emergencies.

(i) Master Schedule Summary

The table below summarizes the standard shift windows and primary documentation obligations for each role. All shifts overlap by 15 minutes to support a structured shift-change huddle and controlled-substance count.

Role	Standard Shift	Coverage	Primary Documentation
Qualified Professional (QP)	7a-9:30p (float)	On-site + on-call 24/7	Service notes, PCP addendums, Form 9, IRIS reports
Associate Professional (AP)	7a-11p (rotating)	1:4 day/evening	Service notes, group documentation, BSP updates
DCP — Day Shift	7a-3p	1:4 youth ratio	Daily logs, MAR, Form 1 handoff, Form 7 notes
DCP — Evening Shift	3p-11p	1:4 youth ratio	Daily logs, MAR, Form 1 handoff, Form 7 notes
DCP — Awake Overnight	11p-7a	1:8 youth ratio	Night Watch Log (Form 1), 15-min room checks, security log
House Manager	9a-5p Mon-Fri	On-site + on-call	Maintenance log, inventory, Form 5 environmental checks
Registered Nurse (RN)	Weekly visit + on-call	On-call 24/7	Nursing notes, MAR updates, delegation training log
Billing Coordinator	8a-4p Mon-Fri	On-site or remote	Per-diem billing, Form 4 suspensions, auth tracking, weekly summary

Shift-Change Huddle. Every shift transition (7:00 AM, 3:00 PM, 11:00 PM) begins with a 15-minute overlap during which the off-going and on-coming staff conduct a controlled-substance count, review any incident reports from the prior shift, and brief on each youth's clinical and behavioral status. The huddle is documented on Form 1 (Shift Change & Awake Night Watch Log).

On-Call Coverage. The QP carries the agency phone 24/7 on a rotating weekly schedule. The RN is on-call 24/7 for medical questions. The House Manager is on-call for facility emergencies during off-hours. The Billing Coordinator is reachable during business hours; the QP handles after-hours billing emergencies. All on-call responses are logged in the on-call binder with timestamp, caller, issue, and resolution.

Deviation Policy. Schedule deviations (call-outs, late arrivals, unplanned overtime) are documented on Form 1. The QP maintains the master staffing schedule and approves any role substitution. Per 10A NCAC 27G .5600, ratios must be maintained at all times; if a replacement is not available, the QP covers in-house until a replacement arrives. Chronic staffing gaps are reported to the Clinical Director per §1.4(a) and may trigger a corrective action plan.

PART 3

Customized Forms & Logs

The following nine forms and logs are the official documentation instruments for daily operations. Each form must be completed in ink or in the electronic record system, signed by the responsible staff member, and filed in the youth's chart or the facility's operational binder as indicated. Originals are retained per the record retention policy (12 years after the minor reaches age 18; 11 years for adults). Forms 7, 8, and 9 are new in Version 2.0 to address RMDM service-note, clinical-record-content, and disclosure-accounting requirements. As of Rev. 2.9, all nine forms are interactive AcroForm fillable PDFs — they are printable, copyable, sharable, editable, and fillable. A Form Properties banner at the top of each form documents these capabilities, and standalone fillable copies are available in the /download/forms/ directory for use outside this manual.

1. Shift Change & Awake Night Watch Log

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 3 · Form 1: Shift Change & Awake Night Watch Log

FORM PROPERTIES

This form is **printable**, **copyable**, **sharable**, **editable**, and **fillable**. Click any field to type. Save the PDF to retain entries. Print to obtain a wet-ink signature. Share the saved or printed copy via secure channels (encrypted email, EHR upload, or sealed envelope). Edit existing entries by clicking the field again. Standalone fillable copies of all forms are in the /download/forms/ directory.

Initial each box to verify you visually saw the youth breathing and in their bed. Click any cell to type.

Facility:

Date:

Awake Overnight Room Checks (Every 15 Minutes)

Time	Youth 1	Youth 2	Youth 3	Youth 4	Staff Initials
11:00 PM					
11:15 PM					
11:30 PM					
11:45 PM					
12:00 AM					
12:15 AM					
12:30 AM					
12:45 AM					
1:00 AM					
1:15 AM					
1:30 AM					
1:45 AM					
2:00 AM					
2:15 AM					

Time	Youth 1	Youth 2	Youth 3	Youth 4	Staff Initials
2:30 AM					
2:45 AM					
3:00 AM					
3:15 AM					
3:30 AM					
3:45 AM					
4:00 AM					
4:15 AM					
4:30 AM					
4:45 AM					
5:00 AM					
5:15 AM					
5:30 AM					
5:45 AM					
6:00 AM					
6:15 AM					
6:30 AM					
6:45 AM					

Shift Handoff Verification: Controlled substance count verified.

Off-Going Staff:

On-Coming Staff:

2. Contraband & Belongings Inventory

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 3 · Form 2: Contraband & Belongings Inventory

FORM PROPERTIES

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Youth Name:

Service Record # / MID:

Date:

Search Type:

Admission

Return from Pass

Probable Cause

QP Approval:

Item Description	Quantity	Brought In / Found	Disposition (Kept / Safe / Guardian)	Staff Initials

Item Description	Quantity	Brought In / Found	Disposition (Kept / Safe / Guardian)	Staff Initials

Youth Acknowledgment: Belongings inventoried in my presence.

Youth Signature:Staff Signature:

3. Physical Restraint & Debriefing Checklist

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 3 · Form 3: Physical Restraint & Debriefing Checklist

FORM PROPERTIES

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Youth
:

Service Record # / MID:

Date:

Time
Started:

Time
Ended:

Total Duration
(Min):

Techniq
ue:

Pre-Restraint De-escalation Attempts:

Verbal
redirection

Offered
break

Sensory
item

BSP
coping
skill

Separation

Reason for Restraint (Imminent danger):

Danger to Self

Danger to Others

Property
destruction
posing risk

Describe objectively:

Post-Restraint Medical Check (Within 1 hour):

Youth checked for injuries, breathing normally:

Yes

No (Seek medical
attention)

Staff Signature:

Notifications:

On-Call QP Notified:

Y

N

Time:

Guardian Notified:

Y

N

Time:

IRIS Report Filed:

Y

N

IRIS ID #:

Post-Restraint Debriefing (Within 24 hours):

Youth debriefed by:

Date/Time:

Trigger?

Youth alternative?

Staff alternative?

Youth Signature:

QP Signature:

4. Home Pass & Medicaid Billing Exclusion Tracker

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 3 · Form 4: Home Pass & Medicaid Billing Exclusion Tracker

FORM PROPERTIES

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Youth
h:

Service Record # /
MID:

Month/Y
ear:

Date Left	Time Left	Destination / Pass	Date Returned	Time Returned	Total Hours Away	Billing Action (Suspension Days)	Staff Initials

Billing Coordinator Sign-Off: Medicaid billing adjusted for suspension days.

Signature:

Date:

5. Emergency Drill & Environmental Safety Log

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 3 · Form 5: Emergency Drill & Environmental Safety Log

FORM PROPERTIES

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Facility:

Monthly Fire Drills (Under 3 minutes)

Month	Date	Time of Day	# Youth	Evac Time	Staff	Signature
Jan						
Feb						
Mar						
Apr						
May						
Jun						
Jul						
Aug						
Sep						
Oct						
Nov						
Dec						

Quarterly Tornado Drills

Quarter	Date	# Youth	Staff	Signature
Q1				
Q2				
Q3				
Q4				

Monthly Environmental Checks

Month	Smoke Detectors (Y/N)	Extinguisher (Y/N)	Hot Water ($\leq 120^{\circ}\text{F}$)	Fridge ($< 40^{\circ}\text{F}$)	Staff Initials
Jan					
Feb					
Mar					
Apr					

Month	Smoke Detectors (Y/N)	Extinguisher (Y/N)	Hot Water ($\leq 120^{\circ}\text{F}$)	Fridge ($< 40^{\circ}\text{F}$)	Staff Initials
May					
Jun					
Jul					
Aug					
Sep					
Oct					
Nov					
Dec					

6. Employee SOP Acknowledgment

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 3 · Form 6: Employee SOP Acknowledgment

FORM PROPERTIES

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Employee Name:

Title:

QP

AP

Direct Care
Professional

By signing below, I acknowledge that I have received, read, and understand the SOP Manual for **Well Spring Intervention LLC** (Rev. 2.9, July 2026, RMDM-Compliant). I understand these policies are mandated by NC DHSR (10A NCAC 27G), NC Medicaid (CCP 8C), Rule 108 (10A NCAC 27T), the NCDHHS Records Management and Documentation Manual (Effective July 8, 2025), NCGS Chapter 66 Article 40 (NC UETA) and the federal E-SIGN Act governing electronic signatures. I understand that the QP reports to the Clinical Director and is responsible for scheduling clinical services, assessments, PCPs, and day-to-day supervision of APs and DCPs according to the Clinical Director's direction, and provides recurring compliance reports to the Clinical Director as defined in §1.4 and §1.4(a). I acknowledge that QP credentialing requirements are specified in §1.4(b) per 10A NCAC 27G .0104 — there are two acceptable pathways: Pathway 1 (master's degree + recognized NC credential + 1 year post-master's supervised MH/DD/SA experience) or Pathway 2 (bachelor's degree + 2 years full-time pre- or post-bachelor's supervised MH/DD/SA experience); a QP is not required to hold a full, unrestricted clinical license. I agree to follow these protocols exactly, including the electronic-signature safeguards and system-unavailability procedures in §10.7. I understand failure to do so may result in disciplinary action, termination, or legal consequences regarding Medicaid fraud and regulatory non-compliance.

Employee Signature:

Date:

QP / Supervisor Signature:

Date:

7. Full Service Note Template (Mandatory)

Reference: RMDM Chapter 6 — Contents of a Full Service Note · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 3 · Form 7: Full Service Note Template

FORM PROPERTIES

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This template must be used for all shift notes. Backdating is prohibited. Photocopying or repeating notes verbatim from a prior date or another individual's record is strictly prohibited. Click any cell in the Content column to type.

Field	Content
Youth Name	
Service Record # / MID	
Date of Service	
Type of Contact	
Place of Service	
Shift / Coverage Hours	
Staff Present (for ratios)	
Purpose / ISP Goal Addressed	
Interventions Provided	
Effectiveness & Youth Response	
Signature / Credentials / Date	
Late Entry (if applicable)	

8. Comprehensive Clinical Record Content Checklist

Reference: RMDM Chapter 2 — Full Clinical Service Records · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 3 · Form 8: Comprehensive Clinical Record Content Checklist

FORM PROPERTIES

This form is **printable, copyable, sharable, editable, and fillable**. Click any field to type. Save the PDF to retain entries. Print to obtain a wet-ink signature. Share the saved or printed copy via secure channels (encrypted email, EHR upload, or sealed envelope). Edit existing entries by clicking the field again. Standalone fillable copies of all forms are in the `/download/forms/` directory.

To be maintained in each youth's chart and reviewed quarterly by the QP. Mark each element Present (Y), Absent (N), or N/A, with date verified. Click any cell in the Present? or Notes columns to type.

Element	Present? (Y/N/Date)	Notes
Treatment Consent		
PCP Consent		
Restrictive Intervention Consent (if applicable)		
Emergency Care Consent		
HIPAA Notice Acknowledgement		
Third-Party Release		
ROI(s)		
Demographics (name, DOB, contact, etc.)		
Emergency Contact / Preferred Physician / Hospital		
Advance Directives (or notation of none)		
Medication Allergies / Adverse Reactions / None		
Health / Behavioral Health History		
DSM-5-TR Diagnosis / ICD-10		
Medication Orders / MAR		
Lab Results (if applicable)		
Notification of Rights / Explanation		
Restrictive Intervention Documentation (if applicable)		
CCA (by licensed professional)		
ASAM Level of Care (if SUD)		
PCP / Service Plan / Crisis Plan		
Service Order (signed)		
Discharge Plan / Summary		
Accounting of Disclosures Log		
42 CFR 2.22 Summary (for SUD)		
Guardianship / POA / Legal Documents		
Incoming/Outgoing Correspondence		
Service Notes (per shift, compliant)		
Incident Occurrence Notations (reports filed separately)		

QP Quarterly Audit Signature:

Date:

9. Accounting of Disclosures Log

Reference: RMDM Chapter 3 — Documentation Requirements when Disclosing Information · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 3 · Form 9: Accounting of Disclosures Log

FORM PROPERTIES

This form is **printable**, **copyable**, **sharable**, **editable**, and **fillable**. Click any field to type. Save the PDF to retain entries. Print to obtain a wet-ink signature. Share the saved or printed copy via secure channels (encrypted email, EHR upload, or sealed envelope). Edit existing entries by clicking the field again. Standalone fillable copies of all forms are in the /download/forms/ directory.

Maintain for minimum 6 years per HIPAA and RMDM requirements. One log per youth. All disclosures of SUD treatment information must additionally comply with 42 CFR Part 2. Click any cell to type.

Youth
Name:

Service Record # / MID:

Date of Disclosure	Recipient (Agency/Individual)	Purpose of Disclosure	Description of Info Disclosed	Disclosing Staff (Name/Title)	Authorization / Exception Basis

10. Version History

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.9, Jul 2026 — RMDM-Compliant) · Part 3 · Version History

Versio n	Date	Summary of Changes	Author
1.0	Jan 2026	Original SOP.	Executive Director / QP

Version	Date	Summary of Changes	Author
2.0	Jul 2026	Updated to full RMDM (July 2025) compliance: Added CDW/data reporting (§1.5); revised record retention to 12 years post-majority (§1.6); added CCA requirements & ASAM (§4.1); added medical necessity (§4.2); added service orders incl. verbal/72-hour (§4.4); added service authorization (§4.5); expanded service note content (§10.1-10.4); added alterations policy (§10.5); added authentication incl. initials/ADA/rubber stamps (§10.6); added administrative closure (§3.5); added accounting of disclosures (§11.4); added privacy/security/42 CFR Part 2 (§11); added individual access (§11.5); added transporting records (§11.6); added grids/modified notes (§10.4); added TB screening (§6.2); added comprehensive clinical record checklist (Form 8); added Full Service Note Template (Form 7); added Accounting of Disclosures Log (Form 9); added Protocol 20 (Service Orders & Auth) and Protocol 21 (Record Management).	Executive Director / QP
2.1	Jul 2026	Auditor recommendation: Added §10.7 Electronic Signatures policy explicitly referencing NCGS Chapter 66, Article 40 (NC Uniform Electronic Transactions Act) and the federal E-SIGN Act (15 U.S.C. § 7001 et seq.). §10.7(a) specifies required administrative, technical, and physical safeguards (unique credentials, MFA, session timeout, TLS/encryption, immutable audit trails, RBAC, immediate revocation, annual review). §10.7(b) establishes System Unavailability Procedures, including paper-fallback handwritten signatures with date/credentials, QP-declared Documentation Continuity Event, 72-hour transcription into EHR with late-entry notation, paper source-document retention, 7-business-day QP review for delayed transcription, and a Documentation Continuity Event log. Prior §10.7 Service Authorizations and §10.8 Billing renumbered to §10.8 and §10.9 respectively.	Executive Director / QP
2.2	Jul 2026	Organizational clarification: §1.4 rewritten to separate the Clinical Director role (overall clinical program responsibility) from the Qualified Professional (QP) role. The QP now reports to the Clinical Director, supervises staff according to the Clinical Director's direction, and escalates clinical, staffing, and quality-of-care concerns. New §1.4(a) QP Compliance Reporting to the Clinical Director defines seven recurring compliance report types: monthly service-note audit summaries, monthly IRIS incident-report status, quarterly Clinical Record Content Checklist audits (Form 8), quarterly Accounting of Disclosures reviews (Form 9), quarterly personnel-file audits, annual electronic-signature safeguard review (§10.7(a)), and ad-hoc immediate reporting of breaches, complaints, licensing visits, and Medicaid audits. Clinical Director signs acknowledgment of each report and directs corrective action.	Executive Director / QP
2.3	Jul 2026	QP responsibilities clarification: §1.4 updated to explicitly include "scheduling" as a QP responsibility. The QP is now responsible for scheduling clinical services, assessments, and PCPs (in addition to clinical services, assessments, PCPs themselves and day-to-day AP/DCP supervision), according to the direction of the Clinical Director. Aligns the SOP with the operational reality that the QP coordinates and schedules all clinical appointments, assessment windows, and Person-Centered Plan meetings for each youth in care.	Executive Director / QP
2.4	Jul 2026	QP credentialing correction (10A NCAC 27G .0104): New §1.4(b) QP Credentialing Requirements added — specifies that a QP is NOT required to hold a full, unrestricted clinical license, but a bachelor's degree alone is not sufficient. Acceptable NC credentials listed across four categories: (i) full clinical license (LCSW, LPC, LMFT, Licensed Psychologist, LPA, psychiatrist MD/DO); (ii) associate/provisional license (LCSW-A, LPC-A, LMFT-A, LCAS-P); (iii) certification (LCAS, CCS with master's, CMSW); or (iv) psychiatric nursing credential (CNS or NP with psychiatric/mental health certification). Plus master's degree in human services, plus one year of full-time post-master's supervised experience, plus NC-DHHS QP training modules. QP must report any lapse/sanction/restriction to the Clinical Director within one business day. §2.2 Staff Qualifications QP bullet updated to match (previous bullet incorrectly stated "Master's or Bachelor's degree... licensed or license-eligible" — a bachelor's degree alone is not 27G .0104-compliant).	Executive Director / QP

Version	Date	Summary of Changes	Author
2.5	Jul 2026	QP credentialing correction (continued): Rev. 2.4 incorrectly removed the bachelor's-degree pathway to QP status. Rev. 2.5 restores BOTH pathways per 10A NCAC 27G .0104: §1.4(b) restructured into Pathway 1 (master's degree in human services + recognized NC credential [full license / associate-provisional license / certification / psychiatric nursing credential] + 1 year full-time post-master's supervised MH/DD/SA experience) AND Pathway 2 (bachelor's degree in a human services field + 2 years full-time pre- or post-bachelor's supervised MH/DD/SA experience, documented by supervising QP, with practice under clinical supervision of a Pathway 1 QP or the Clinical Director until NC-DHHS QP training is completed). Common Requirements subsection added covering QP training, lapse/sanction reporting, personnel-file verification, and annual re-verification of pathway. §2.2 QP bullet updated to list both pathways.	Executive Director / QP
2.6	Jul 2026	Cover redesign: Added a symbolic cover illustration (rendered banner between the kicker and the entity title) evoking empowerment, growth, freedom, health, wholeness, and healing — visually framing the trauma-informed, restorative mission of the program. The illustration depicts a stylized tree-human figure rising toward a sunrise over calm water, rendered in a warm earthy palette (terracotta, warm brown, soft rose, cream) consistent with the manual's existing cascade palette. A small caption strip below the image reads "Empowerment · Growth · Freedom · Health · Wholeness · Healing". No body-content changes; §1.4(b) QP Credentialing Requirements (Pathway 1 and Pathway 2) and all other SOPs, protocols, and forms remain unchanged from Rev. 2.5.	Executive Director / QP
2.7	Jul 2026	Cover redesign (continued): Cover is now a full-bleed brand illustration suitable for reuse across company websites, publications, and collateral materials. A new high-resolution 768×1344 portrait rendering of the tree-human-sunrise image fills the entire cover page. Overlay text is reduced to the essentials: company name (Playfair Display 72pt white with text-shadow), tagline ("Empowerment · Growth · Freedom · Health · Wholeness · Healing"), subtitle ("Level 3 Supervised Residential Group Home"), and a bottom document-identification panel (Doc. WSI-SOP-001, Effective July 2026, RMDM-Compliant, Owner, Revision 2.7). All descriptive content previously on the cover (full summary paragraph, Population/Served/Service Type/Effective Date/Owner meta block, and the Regulatory Framework reference block) is relocated to a new About This Manual page (p. 2 of the body, p. 3 of the final merged PDF) immediately preceding the Table of Contents. The About This Manual page also includes a Revision Lineage summary and a Cover Artwork note describing the brand visual's intended reuse. TOC page-number offset increased from +1 to +2 to account for the new inside page. Body content (SOPs, protocols, forms, §1.4(b) QP Credentialing Requirements) is unchanged from Rev. 2.6.	Executive Director / QP
2.8	Jul 2026	Cover redesign (continued): Per organizational direction that versioning is private, all versioning information has been removed from the public-facing cover. The bottom band of the cover previously displayed Doc. WSI-SOP-001, Effective July 2026 · RMDM-Compliant, Owner: Executive Director & Qualified Professional (QP), and a large "Revision 2.7" display — all of these elements have been removed. The cover now displays only the brand-essential content: company name (Well Spring Intervention LLC), service-type subtitle (Level 3 Supervised Residential Group Home), values tagline (Empowerment · Growth · Freedom · Health · Wholeness · Healing), the full horizontal brand illustration, and a document-type label ("SOP & Operational Manual / Standard Operating Procedures, Protocols & Forms") with an accent rule. Versioning information remains fully accessible on internal surfaces only: PDF metadata (/Title, /Subject, /Keywords), the About This Manual inside page (p. 2), body page headers (every body page footer shows Doc ID and Rev. 2.8), the Version History table in Part 3, and Form 6 (Employee SOP Acknowledgment). Body content (SOPs, protocols, forms, §1.4(b) QP Credentialing Requirements) is unchanged from Rev. 2.7.	Executive Director / QP

Version	Date	Summary of Changes	Author
2.9	Jul 2026	Three-part enhancement. (1) Cover artwork refresh: the brand illustration is regenerated with lush GREEN leaves (replacing the prior amber/gold leaves) to more vividly symbolize growth, and an explicit well-spring (a circular pool of fresh water with concentric ripples) is added to the foreground directly in front of the tree-human figure, completing the symbolic narrative of wellspring, growth, health, and flourishing. The composition is otherwise unchanged: stylized tree-human figure, warm sunrise over calm water, warm earthy palette, horizontal 1344x768 aspect ratio, text-free. The standalone brand PNG (Well_Spring_Brand_Image_1344x768.png) is regenerated in lockstep. (2) New Protocol 22, Daily Workflow Schedules for All Personnel: codifies time-blocked daily routines for every personnel classification (QP, AP/PP, DCP Day Shift, DCP Evening Shift, DCP Awake Overnight, House Manager, RN, Billing Coordinator), with a master schedule summary table and explicit shift-change huddle / on-call / deviation policies. Aligns the SOP with the operational reality that the QP coordinates and schedules all clinical appointments and supervision across all shifts. (3) Forms enhancement, all nine forms in Part 3 are converted to interactive AcroForm fillable PDF fields. Each form now opens with a Form Properties banner declaring it printable, copyable, sharable, editable, and fillable. AcroTextField flowables replace underscore blanks; AcroCheckbox flowables replace bracket-checkboxes; signature and metadata lines use new fillable_meta_row / fillable_check_row / fillable_signature_row helpers. Standalone fillable PDF copies of all nine forms are also generated into /download/forms/ for use outside this manual. The Form 7 Service Name row (previously hardcoded to "Level 3 Residential, Shift") is removed in favor of fully fillable rows. Body content (SOPs §1-§11, Protocols 1-21, §1.4(b) QP Credentialing Requirements) is otherwise unchanged from Rev. 2.8.	Executive Director / QP